Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0006	**Age / Sex**	31 / M
Setting	Emergency Dept	**Primary dx**	Opioid overdose

Chief complaint

Unresponsive in a public restroom, bystander-initiated CPR not required.

History of present illness

Found slumped with a syringe beside him. EMS found RR 6/min, SpO2 82%, pinpoint pupils. Bag-valve-mask ventilation, naloxone 0.4 mg IM with partial response en route.

Past history / medications

History of opioid use disorder, hepatitis C. No known allergies.

Examination

RR 8, shallow. SpO2 94% on oxygen. HR 62, BP 104/62. Pupils 1 mm bilaterally. Fresh injection marks left antecubital fossa. No focal neurology.

Glasgow Coma Scale

No eye opening. Moans without words. Motor: withdraws purposefully from pain, does not follow commands.

GCS on arrival	Score	Response
Eye opening (E)	1	None
Verbal (V)	2	2 - Incomprehensible sounds
Motor (M)	4	Withdraws from pain
TOTAL GCS		**7**

GCS after naloxone	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**14**

Investigations

Glucose 5.8 mmol/L. VBG respiratory acidosis (pH 7.24, pCO2 62 mmHg). Urine drug screen positive for opiates and fentanyl. Paracetamol level undetectable. CK 640.

Assessment

Opioid toxidrome with respiratory depression, GCS 7. Naloxone-responsive.

Plan / disposition

Naloxone titrated 0.4 mg IV with GCS recovery to 14 and RR 16, followed by infusion due to long-acting opioid concern. Monitored 6 h after last dose. Addiction service consult, take-home naloxone provided.